Something Global This Way Comes — And It's Not Just About Health

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JASON IAN POBLETE

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Tedros Adhanom Ghebreyesus (on screen), the Director-General of the World Health Organization, delivers a message to the multi-stakeholder hearing on the prevention and control of noncommunicable diseases (NCDs) and the promotion of mental health and well-being. Source: United Nations UN71096180.

Earlier this week, I sat down with a friend and fellow international legal practitione Professor Jane Adolphe. Many of you know her as the former international human rights advisor to the Holy See's Secretariat of State, and a respected law professor a Ave Maria School of Law. I've collaborated with Jane through the International Catholic Jurist Forum, where we regularly discuss the complex and often subtle shi in global governance, human rights law, and religious freedom.

Our conversation this time focused on something urgent—something not unfoldin Washington or Rome, but thousands of miles away in Geneva, Switzerland. In fact, this weekend, the World Health Organization (WHO) is convening its 78th World Health Assembly to finalize and vote on two seemingly technocratic, but profoundl consequential, international documents: a new *Pandemic Agreement* and a package c amendments to the *International Health Regulations (IHR)*.

The U.S. Department of Health and Human Services (HHS) maintains a page explainin America's role in the International Health Regulations (IHR), including what they are, hc they work, and what obligations the U.S. has already accepted. What is a U.S. IHR Natio Focal Point (NFP)? Follow this link.

"So, great Jason," you may be asking, "what does this have to do with me here in America?" A lot more than you think, and our Congress needs to get off the dime a start to do something about it.

On paper, global WHO bureaucrats promise better coordination in the event of glo health crises. Remember the last global health crisis? COVID? In practice, these

unelected officials risk shifting extraordinary power to even more unelected international bureaucrats at the expense of American national sovereignty, individu freedom, property rights, and the moral architecture of subsidiarity that undergirds just governance and the rule of law.

You may recall that President Trump announced, on January 20 of this year, that the United States would formally withdraw from the WHO. This decision, rightly made was welcomed by those of us who have long warned that the WHO had strayed from its original mission of public health and entered the dangerous realm of global political management. However, as Jane and I discussed over the past few months—and as I've since reviewed more carefully through legal literature and congressional research—withdrawal from the WHO is not automatic, and at the rate we are going Washington, DC, it may never happen.

Under international law, specifically the Vienna Convention on the Law of Treaties withdrawal from a treaty that contains no exit clause requires a full twelve months' notice. Since the WHO Constitution includes no clear formal withdrawal provision the U.S. remains a full member until at least January 2026—regardless of the execut order issued earlier this year. That means we remain subject to the very agreements now being negotiated in Geneva.

But it gets more legal and politically complicated.

Under U.S. law, Congress imposed additional conditions when it authorized our participation in the WHO in 1948. That statute clarifies that even after notice is giv the United States must first pay its full financial obligations to the WHO for the current fiscal year. If it doesn't, the withdrawal may not take effect at all.

Congress, therefore, must act affirmatively—not only to fund the exit, but to ensure occurs within the legal bounds of our own system. If it fails to do so, we may find ourselves still legally entangled in WHO governance mechanisms well into 2026, ev as other nations begin implementing a new global health regime.

And that is what's so concerning.

While the United States and Argentina have signaled their intent to withdraw, the r of the world is marching ahead. History reminds us that even when one administrat tries to pull us out of an international organization, another might sign us back in. was not just Trump. Presidents Gerald Ford and Ronald Reagan also tried—Ford wi the International Labor Organization, Reagan with UNESCO.

Even if Congress manages to fully sever American participation in the WHO before the midterm elections, who will leave with us? Argentina? That's it. WHO Director General Tedros, a Communist backed by China and other globalist power centers, i already reported to be raising funds to cover the $130 million annual shortfall the U exit would create.

Even so, the Trump administration has left the door open to negotiation. Earlier thi year in Las Vegas, President Trump said, "We would have to clean it up a little bit." Tedros has reportedly begun reforming the *process*, but not the core *substance* of the Pandemic Treaty or the IHR amendments. And that distinction matters. These are r transparency tweaks. These are profound structural shifts.

Most Americans don't know that the U.S. already adopted the IHR long ago. Nor, frankly, do most members of Congress or their staff—at least not the ones I've raise this issue with.

These WHO instruments—the Pandemic Treaty and the IHR amendments—contai language that empowers the WHO to declare global health emergencies and coordinate binding responses, even over the objection of sovereign nations. That m include border controls, digital health certificates, vaccine mandates, and coordinat speech "management" during so-called *infodemics*. None of this aligns with the principle of subsidiarity. Nor does it honor the dignity of the human person, whose rights derive not from bureaucratic consensus, but from natural law and divine ord

The WHO claims these are only recommendations. But we've seen how recommendations from international bodies quickly become binding in practice through trade requirements, digital platforms, or corporate enforcement. Once bak into global frameworks, these policies often escape the control of the governments that agreed to them.

Soft law becomes hard governance.

Even worse, as of today, the U.S. government has not only failed to stop this proces it has made no serious attempt to **persuade** other countries to slow down, question, oppose it. There have been hearings in Congress, and a few bills introduced—one o them would require Senate approval for any such treaty. But not a single one has reached the floor.

In a moment of tremendous consequence, Washington is distracted, fragmented, ar largely silent.

We should not be.

This issue is not about partisanship. It is about principle. It is about the long arc of campaign to remove decision-making power from the family, the parish, and the lo community, and reassign it to global committees unmoored from faith, culture, or democratic accountability. It is about turning natural rights—like medical privacy, bodily autonomy, and the freedom to travel—into negotiable permissions managed remote bureaucracies.

Catholic teaching, rooted in truth and ordered liberty, has always stood against this kind of centralization. The principle of subsidiarity is not a convenience. It is a mor safeguard against tyranny. It insists that decisions should be made as close to the person as possible—by families, communities, and local authorities—not by technocrats half a world away.

Yet here we are, watching a new global infrastructure rise—this time under the ban of "health."

So what is to be done?

Congress must act. Not later. Not after the election. Now.

The window is open, but it's rapidly narrowing. Lawmakers must pass binding legislation to prohibit the WHO pandemic mandates from being enforced within th United States. They must affirm that no international agreement may override U.S. sovereignty without Senate ratification. They must also appropriate the funds need to ensure that the U.S. exit from the WHO is legally complete.

If the Trump administration intends to follow through, it should also begin diplom work now to encourage other nations to join us, not after the vote, but before. Righ now, it's just us and Argentina. And if the rest of the world embraces this new globa model, America will be left isolated, while a new health bureaucracy—and data governance superstructure—is constructed around us. It's not a matter of if it will reach us, but when.

As I've said before, and as Jane reminded me again this week, someone else will wri the rules for us if we do not guard our freedoms. Congress should work quickly witl President Trump and not wait until January 2026 to find out that the rules have alre changed.

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